



Cardiac Arrest: ROSC and Induced Hypothermia Post-Resuscitation Care

Assessment

Pediatric Pearls:

- Use pediatric dosing of medications or electrical therapy for a pediatric patient < 37 kg and as defined by the PEDIA Tape.
- Focus on rapid and early BLS airway and ventilation tools. Intubation may not be the best option for these patients.

Signs & Symptoms:

- Return of pulse from a Non-Traumatic Cardiac Arrest

Differential:

- Continue to address specific differentials associated with original dysrhythmia.

Clinical Management Options

P	P	P	P	P	P	• Continue Oxygenation, target SpO₂ 92% - 96%
L	L	L	L	L	L	• Use Post Resuscitation Checklist as indicated
1	2	3	4	5	6	• As time allows, expose patient and apply ice packs to axilla, neck, and groin
						• Vascular access IO can be placed first, but IV access is preferred when able to obtain. If patient ≤ 12 months, distal femur IO is preferred • Fluid bolus with isotonic crystalloid as needed (max 1000mL unless suspect hypovolemia)
						• Vasopressor infusion titrated to MAP = 80 mmHg ○ Norepinephrine preferred in adult patients ○ Epinephrine preferred in pediatric patients • Advanced Airway Management as needed • Monitor and interpret ECG • Resuscitation Alert if not already done so • STEMI activation if appropriate and transmit 12-lead • Midazolam or Ketamine for sedation as needed • Rocuronium <i>only</i> after advanced airway placement • Target normothermia (temperature < 38°C)

Consult Online Medical Control as Needed

Pearls:

- Providers should not hasten departure after obtaining ROSC. Instead, focus on stabilizing the patient and ensuring that the airway is appropriately managed. If rearrest occurs, it will be in the first few minutes.
- PL6 providers - consider [Rapid Sequence Induction](#) with [Rocuronium](#) after adequate resuscitation, use [Airway management checklist](#)
- If patient is hypotensive do not administer sedative/paralytic. Initiate volume replacement with isotonic crystalloid and vasopressors as needed.
- When exposing patient for purpose of cooling, undergarments may remain in place to preserve the patient's modesty.
- Reassess airway frequently and with every patient move.
- Patients develop metabolic alkalosis with cooling. Do not hyperventilate.
- These patients should only be transported to designated Resuscitation Centers; refer to [Cardiac Transport Grid](#).